



Patient Information	Specimen Information	Client Information
HANEY, MATTHEW DOB: 04/26/1993 AGE: 33 Gender: M Fasting: Y Phone: 805.443.0290 Patient ID: 5467CB51 Health ID: 8573004336378437	Specimen: ZD835935N Requisition: 0346247 Lab Ref #: BDE26F26D5C2480A Collected: 08/06/2026 / 10:41 PDT Received: 08/06/2026 / 19:36 PDT Reported: 08/22/2026 / 13:28 PDT	Client #: 73929412 MAIL992 DAMASCO, LEO JUNCTION 440 N BARRANCA AVE # 3811 COVINA, CA 91723-1722

COMMENTS: PATIENT MUST BE FASTING
FASTING: YES

Test Name	In Range	Out Of Range	Reference Range	Lab
IRON, TIBC AND FERRITIN PANEL				
IRON AND TOTAL IRON				EN
BINDING CAPACITY				
IRON, TOTAL	52		50-180 mcg/dL	
IRON BINDING CAPACITY	338		250-425 mcg/dL (calc)	
% SATURATION		15 L	20-48 % (calc)	
FERRITIN	76		38-380 ng/mL	EN
ALBUMIN, RANDOM URINE W/CREATININE				
CREATININE, RANDOM URINE	291		20-320 mg/dL	EN
ALBUMIN, RANDOM URINE				EN
W/CREATININE				
ALBUMIN, URINE	0.7		mg/dL	
	Reference Range			
	Not established			
ALBUMIN/CREATININE RATIO, RANDOM URINE	2		<30 mg/g creat	

The ADA defines abnormalities in albumin excretion as follows:

Albuminuria Category	Result (mg/g creatinine)
Normal to Mildly increased	<30
Moderately increased	30-299
Severely increased	> OR = 300

The ADA recommends that at least two of three specimens collected within a 3-6 month period be abnormal before considering a patient to be within a diagnostic category.

METHYLMALONIC ACID AND HOMOCYSTEINE			
METHYLMALONIC ACID	157	55-335 nmol/L	EZ

Serum methylmalonic acid (MMA) levels are used to diagnose and monitor several rare inborn errors of metabolism, including methylmalonic aciduria. The enzymatic conversion of MMA to succinic acid requires vitamin B12 (adenosyl-cobalamin) as a cofactor. Serum MMA levels are also used for assessing functional vitamin B12 deficiency. Vitamin B12 is essential for fetal neurodevelopment, particularly early in pregnancy. Undiagnosed maternal vitamin B12 deficiency may be associated with adverse fetal/neonatal outcomes, such as neural tube defects and intrauterine growth restriction.

Quest Diagnostics utilized Multi-Modal Decomposition (MMD) analysis to establish first and second trimester-specific MMA reference intervals in pregnancy, as given below:

MMA, First trimester (<13 wks gestation): 58-167 nmol/L
MMA, Second trimester (13-23 wks gestation): 63-241 nmol/L



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Test Name	In Range	Out Of Range	Reference Range	Lab
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HOMOCYSTEINE	14.7 H	< OR = 13.5 umol/L	EZ
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Homocysteine is increased by functional deficiency of folate or vitamin B12. Testing for methylmalonic acid differentiates between these deficiencies. Other causes of increased homocysteine include renal failure, folate antagonists such as methotrexate and phenytoin, and exposure to nitrous oxide. Selhub J, et al. Ann Intern Med. 1999;131(5):331-9.

TESTOSTERONE, FREE, BIOAVAILABLE AND TOTAL, MS			
ALBUMIN	4.5	3.6-5.1 g/dL	EZ
SEX HORMONE BINDING GLOBULIN	18	10-50 nmol/L	EZ
TESTOSTERONE, TOTAL, MS	349	250-1100 ng/dL	EZ

Men age 18 or above, with clinically significant hypogonadal symptoms and total testosterone values repeatedly in the range of 200-300 ng/dL or less, may benefit from testosterone treatment after adequate risk and benefits counseling.

For additional information, please refer to <http://education.questdiagnostics.com/faq/TotalTestosteroneL> CMSMS (This link is being provided for informational/educational purposes only.)

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TESTOSTERONE, FREE AND BIOAVAILABLE			EZ
TESTOSTERONE, FREE	70.5	46.0-224.0 pg/mL	

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TESTOSTERONE, BIOAVAILABLE	145.0	110.0-575.0 ng/dL	
HS CRP	3.6 H	mg/L	EN

Reference Range
Optimal <1.0
Jellinger PS et al. Endocr Pract.2017;23(Suppl 2):1-87.
For ages >17 Years:
hs-CRP mg/L Risk According to AHA/CDC Guidelines
<1.0 Lower relative cardiovascular risk.
1.0-3.0 Average relative cardiovascular risk.
3.1-10.0 Higher relative cardiovascular risk.



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>10.0

Consider retesting in 1 to 2 weeks to exclude a benign transient elevation in the baseline CRP value secondary to infection or inflammation. Persistent elevation, upon retesting, may be associated with infection and inflammation.

Pearson TA, Mensah GA, Alexander RW, et al. Markers of inflammation and cardiovascular disease: application to clinical and public health practice: A statement for healthcare professionals from the Centers for Disease Control and Prevention and the American Heart Association. Circulation 2003; 107(3): 499-511.

COMPREHENSIVE METABOLIC

EN

PANEL		
GLUCOSE	82	65-99 mg/dL
		Fasting reference interval
UREA NITROGEN (BUN)	12	7-25 mg/dL
CREATININE	1.07	0.60-1.26 mg/dL
EGFR	94	> OR = 60 mL/min/1.73m2
BUN/CREATININE RATIO	SEE NOTE:	6-22 (calc)
	Not Reported: BUN and Creatinine are within reference range.	
SODIUM	141	135-146 mmol/L
POTASSIUM	4.6	3.5-5.3 mmol/L
CHLORIDE	103	98-110 mmol/L
CARBON DIOXIDE	24	20-32 mmol/L
CALCIUM	9.9	8.6-10.3 mg/dL
PROTEIN, TOTAL	7.3	6.1-8.1 g/dL
ALBUMIN	4.4	3.6-5.1 g/dL
GLOBULIN	2.9	1.9-3.7 g/dL (calc)
ALBUMIN/GLOBULIN RATIO	1.5	1.0-2.5 (calc)
BILIRUBIN, TOTAL	0.4	0.2-1.2 mg/dL
ALKALINE PHOSPHATASE	75	36-130 U/L
AST	15	10-40 U/L
ALT	15	9-46 U/L
HEMOGLOBIN A1c	5.2	<5.7 %

EN

For the purpose of screening for the presence of diabetes:

<5.7% Consistent with the absence of diabetes
5.7-6.4% Consistent with increased risk for diabetes (prediabetes)
> or =6.5% Consistent with diabetes

This assay result is consistent with a decreased risk of diabetes.

Currently, no consensus exists regarding use of hemoglobin A1c for diagnosis of diabetes in children.



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Test Name	In Range	Out Of Range	Reference Range	Lab
According to American Diabetes Association (ADA) guidelines, hemoglobin A1c <7.0% represents optimal control in non-pregnant diabetic patients. Different metrics may apply to specific patient populations. Standards of Medical Care in Diabetes(ADA).				
MAGNESIUM	2.2		1.5-2.5 mg/dL	EN
URIC ACID	5.8		4.0-8.0 mg/dL	EN
Therapeutic target for gout patients: <6.0 mg/dL				
GGT	14		3-90 U/L	EN
AMYLASE	51		21-101 U/L	EN
LIPASE		61 H	7-60 U/L	EN
TSH	1.28		0.40-4.50 mIU/L	EN
T4, FREE	1.5		0.8-1.8 ng/dL	EN
T3, FREE	3.0		2.3-4.2 pg/mL	EN
THYROID PEROXIDASE AND THYROGLOBULIN ANTIBODIES				
THYROGLOBULIN ANTIBODIES	<2		< or = 2 IU/mL	EN
The cutoff value of < or = 2 IU/mL represents the lower limit of quantification (LOQ) established by the assay manufacturer (Beckman Coulter). While very low levels of TgAb may not be clinically significant, any measurable TgAb above LOQ has the potential for interference with Thyroglobulin assay. The manufacturer of the assay found that 96% of the participants in a healthy population (no evidence of thyroid or autoimmune disease) had TgAb levels below 4 IU/mL.				
THYROID PEROXIDASE ANTIBODIES	<1		<9 IU/mL	EN
LEPTIN	4.8		ng/mL	EZ

Reference Ranges for Leptin:

Adult Lean Subjects (18-71 years) with BMI range of 18-25:

Males: 0.3-13.4 ng/mL
Females: 4.7-23.7 ng/mL

Adult Subjects (19-60 years) with BMI range of 25-30:

Males: 1.8-19.9 ng/mL
Females: 8.0-38.9 ng/mL

Pediatric Reference Ranges for Leptin:

5-9.9 years: 0.6-16.8 ng/mL
10-13.9 years: 1.4-16.5 ng/mL
14-17.9 years: 0.6-24.9 ng/mL

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Test Name	In Range	Out Of Range	Reference Range	Lab
has been validated pursuant to the CLIA regulations and is used for clinical purposes.				
CBC (INCLUDES DIFF/PLT)				EN
WHITE BLOOD CELL COUNT	7.7		3.8-10.8 Thousand/uL	
RED BLOOD CELL COUNT	5.54		4.20-5.80 Million/uL	
HEMOGLOBIN	15.9		13.2-17.1 g/dL	
HEMATOCRIT	48.2		39.4-51.1 %	
MCV	87.0		81.4-101.7 fL	
MCH	28.7		27.0-33.0 pg	
MCHC	33.0		31.6-35.4 g/dL	
RDW	13.0		11.0-15.0 %	
PLATELET COUNT	292		140-400 Thousand/uL	
MPV	10.6		7.5-12.5 fL	
ABSOLUTE NEUTROPHILS	4135		1500-7800 cells/uL	
ABSOLUTE LYMPHOCYTES	2379		850-3900 cells/uL	
ABSOLUTE MONOCYTES	493		200-950 cells/uL	
ABSOLUTE EOSINOPHILS		631 H	15-500 cells/uL	
ABSOLUTE BASOPHILS	62		0-200 cells/uL	
NEUTROPHILS	53.7		%	
LYMPHOCYTES	30.9		%	
MONOCYTES	6.4		%	
EOSINOPHILS	8.2		%	
BASOPHILS	0.8		%	
RHEUMATOID FACTOR	<10		<14 IU/mL	EN
CORTISOL, TOTAL	9.7		mcg/dL	EN
Reference Range: For 8 a.m.(7-9 a.m.) Specimen: 4.0-22.0				
Reference Range: For 4 p.m.(3-5 p.m.) Specimen: 3.0-17.0				
* Please interpret above results accordingly *				
DHEA SULFATE	113		93-415 mcg/dL	EN
FSH	3.2		1.4-12.8 mIU/mL	EN
INSULIN		19.0 H	uIU/mL	EN
Reference Range < or = 18.4				
Risk:				
Optimal				< or = 18.4
Moderate				NA
High				>18.4
Adult cardiovascular event risk category cut points (optimal, moderate, high) are based on Insulin Reference Interval studies performed at Quest Diagnostics in 2022.				
LH	4.2		1.5-9.3 mIU/mL	EN
PROLACTIN	3.1		2.0-18.0 ng/mL	EN
ESTRADIOL		40 H	< OR = 39 pg/mL	EN
Reference range established on post-pubertal patient population. No pre-pubertal reference range established using this assay. For any patients for whom low Estradiol levels are anticipated (e.g. males, pre-pubertal children and hypogonadal/post-menopausal females), the Quest Diagnostics Nichols Institute Estradiol, Ultrasensitive, LCMSMS assay is recommended (order code 30289).				



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Please note: patients being treated with the drug fulvestrant (Faslodex(R)) have demonstrated significant interference in immunoassay methods for estradiol measurement. The cross reactivity could lead to falsely elevated estradiol test results leading to an inappropriate clinical assessment of estrogen status. Quest Diagnostics order code 30289-Estradiol, Ultrasensitive LC/MS/MS demonstrates negligible cross reactivity with fulvestrant.

PSA (FREE AND TOTAL)			EN
PSA, TOTAL	1.1	< OR = 4.0 ng/mL	
PSA, FREE	0.4	ng/mL	
PSA, % FREE	36	>25 % (calc)	

PSA(ng/mL)	Free PSA(%)	Estimated(x) Probability of Cancer(as%)
0-2.5	(*)	Approx. 1
2.6-4.0(1)	0-27(2)	24(3)
4.1-10(4)	0-10	56
	11-15	28
	16-20	20
	21-25	16
	>or =26	8
>10(+)	N/A	>50

References: (1)Catalona et al.:Urology 60: 469-474 (2002)
(2)Catalona et al.:J.Urol 168: 922-925 (2002)
Free PSA(%) Sensitivity(%) Specificity(%)
< or = 25 85 19
< or = 30 93 9
(3)Catalona et al.:JAMA 277: 1452-1455 (1997)
(4)Catalona et al.:JAMA 279: 1542-1547 (1998)

(x)These estimates vary with age, ethnicity, family history and DRE results.
(*)The diagnostic usefulness of % Free PSA has not been established in patients with total PSA below 2.6 ng/mL
(+)In men with PSA above 10 ng/mL, prostate cancer risk is determined by total PSA alone.

The Total PSA value from this assay system is standardized against the equimolar PSA standard. The test result will be approximately 20% higher when compared to the WHO-standardized Total PSA (Siemens assay). Comparison of serial PSA results should be interpreted with this fact in mind.

PSA was performed using the Beckman Coulter Immunoassay method. Values obtained from different assay methods cannot be used interchangeably. PSA levels, regardless of value, should not be interpreted as absolute evidence of the presence or absence of disease.

LEAD (VENOUS)	<1.0	<3.5 mcg/dL	EN
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Test Name	In Range	Out Of Range	Reference Range	Lab
No safe blood lead level (BLL) in children has been identified.				

Blood lead levels above 3.5 mcg/dL have been associated with adverse health effects in all age groups. Patient management varies by age and CDC Blood Lead Level range. Refer to the CDC website regarding Lead Publications/Case Management for recommended interventions.

See Endnote 1

MERCURY, BLOOD	<4	<OR=10 mcg/L	EN
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See Endnote 1

ZINC	72	60-130 mcg/dL	EN
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ABO GROUP AND RH TYPE		EN
ABO GROUP	O	
RH TYPE	RH(D) NEGATIVE	

For additional information, please refer to <http://education.QuestDiagnostics.com/faq/FAQ111> (This link is being provided for informational/educational purposes only.)

Endnote 1

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Endocrinology

Test Name	Result	Reference Range	Lab
VITAMIN D,25-OH,TOTAL,IA	35	30-100 ng/mL	EN
Vitamin D Status 25-OH Vitamin D: Deficiency: <20 ng/mL Insufficiency: 20 - 29 ng/mL Optimal: > or = 30 ng/mL For 25-OH Vitamin D testing on patients on D2-supplementation and patients for whom quantitation of D2 and D3 fractions is required, the QuestAssureD(TM) 25-OH VIT D, (D2,D3), LC/MS/MS is recommended: order code 92888 (patients >2yrs). For additional information, please refer to http://education.QuestDiagnostics.com/faq/FAQ199 (This link is being provided for informational/ educational purposes only.) Physician Comments:			

Immunology

Test Name	Result	Reference Range	Lab
ANA SCREEN, IFA, W/REFL TITER AND PATTERN			EN
ANA SCREEN, IFA	NEGATIVE	NEGATIVE	
ANA IFA is a first line screen for detecting the presence of up to approximately 150 autoantibodies in various autoimmune diseases. A negative ANA IFA result suggests an ANA-associated autoimmune disease is not present at this time, but is not definitive. If there is high clinical suspicion for Sjogren's syndrome, testing for anti-SS-A/Ro antibody should be considered. Anti-Jo-1 antibody should be considered for clinically suspected inflammatory myopathies. AC-0: Negative International Consensus on ANA Patterns (https://doi.org/10.1515/cclm-2018-0052) For additional information, please refer to http://education.QuestDiagnostics.com/faq/FAQ177 (This link is being provided for informational/ educational purposes only.) Physician Comments:			



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Cardio IQ®

Test Name	Current		Risk/Reference Interval				Historical
	Result & Risk		Optimal	Moderate	High	Units	Result & Risk
	Optimal	Non-Optimal					
LIPID PANEL							
CHOLESTEROL, TOTAL	166		<200	N/A	>=200	mg/dL	
HDL CHOLESTEROL	47		>=40	N/A	<40	mg/dL	
TRIGLYCERIDES	71		<150	150-199	>=200	mg/dL	
LDL-CHOLESTEROL		103	<100	100-129	>129	mg/dL (calc)	
CHOL/HDL-C RATIO	3.5		<=3.5	3.6-5.0	>5.0	calc	
NON-HDL CHOLESTEROL	119		<130	130-189	>=190	mg/dL (calc)	
LIPOPROTEIN FRACTIONATION, ION MOBILITY							
LDL PARTICLE NUMBER		1197	<1138	1138-1409	>1409	nmol/L	
LDL SMALL		223	<142	142-219	>219	nmol/L	
LDL MEDIUM		244	<215	215-301	>301	nmol/L	
HDL LARGE		4135	>6729	6729-5353	<5353	nmol/L	
LDL PATTERN		B	A	N/A	B	Pattern	
LDL PEAK SIZE		216.8	>222.9	222.9-217.4	<217.4	Angstrom	
APOLIPOPROTEINS							
APOLIPOPROTEIN B	82		<90	90-129	>=130	mg/dL	
LIPOPROTEIN (a)	45		<75	75-125	>125	nmol/L	
FATTY ACIDS							
OmegaCheck® Whole Blood: (EPA+DPA+DHA)		3.0	>=5.5	3.8-5.4	<=3.7	% by wt	
ARACHIDONIC ACID/EPA RATIO		61.5 H	3.7-40.7				



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Test Name	Current		Risk/Reference Interval				Historical
	Result & Risk		Optimal	Moderate	High	Units	Result & Risk
	Optimal	Non-Optimal					
OMEGA-6/OMEGA-3 RATIO	11.7		3.7-14.4				
OMEGA-3 TOTAL	3.0		%				by wt
EPA	0.2		0.2-2.3				% by wt
DPA	0.8		0.8-1.8				% by wt
DHA	2.0		1.4-5.1				% by wt
OMEGA-6 TOTAL	35.3		%				by wt
ARACHIDONIC ACID	12.3		8.6-15.6				% by wt
LINOLEIC ACID	19.6		18.6-29.5				% by wt

For details on reference ranges please refer to the reference range/comment section of the report.

Medical Information For Healthcare Providers: If you have questions about any of the tests in our Cardio IQ offering, please call Client Services at our Quest Diagnostics-Cleveland HeartLab Cardiometabolic Center of Excellence. They can be reached at 866.358.9828, option 1 to arrange a consult with our clinical education team.



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Reference Range/Comments

Analyte Name	In Range	Out Range	Reference Range	Lab
ARACHIDONIC ACID/EPA RATIO		61.5	3.7-40.7	EZ
EPA+DPA+DHA		3.0	>5.4 % by wt	EZ
Relative Risk Categories: Optimal: > or = 5.5 Moderate: 3.8-5.4 High: < or = 3.7 Increasing blood levels of long-chain n-3 fatty acids are associated with a lower risk of sudden cardiac death (1). Based on the top (75th percentile) and bottom (25th percentile) quartiles of the Quest Diagnostics reference population, the following relative risk categories were established for OmegaCheck: A cut-off of > or = 5.5% by wt defines a population at optimal relative risk, 3.8-5.4% by wt defines a population at moderate relative risk, and < or = 3.7% by wt defines a population at high relative risk of sudden cardiac death. The totality of the scientific evidence demonstrates that when consumption of fish oils is limited to 3 g/day or less of EPA and DHA, there is no significant risk for increased bleeding time beyond the normal range. A daily dosage of 1 gram of EPA and DHA lowers the circulating triglycerides by about 7-10% within 2 to 3 weeks. (Reference: 1-Albert et al. NEJM. 2002; 346:1113-1118). This test was developed and its analytical performance characteristics have been determined by Quest Diagnostics. It has not been cleared or approved by the FDA. This assay has been validated pursuant to the CLIA regulations and is used for clinical purposes.				
HDL LARGE		4135	>6729 nmol/L	Z4M
Relative Risk: Optimal >6729; Moderate 6729-5353; High <5353. Male Reference Range: 4334 to 10815 nmol/L; Female Reference Range: 5038 to 17886 nmol/L.				
LDL MEDIUM		244	<215 nmol/L	Z4M
Relative Risk: Optimal <215; Moderate 215-301; High >301. Male Reference Range: 167 to 485 nmol/L; Female Reference Range: 121 to 397 nmol/L.				
LDL PARTICLE NUMBER		1197	<1138 nmol/L	Z4M
Relative Risk: Optimal <1138; Moderate 1138-1409; High >1409. Male and Female Reference Range: 1016 to 2185 nmol/L.				
LDL PATTERN		B	A Pattern	Z4M
Relative Risk: Optimal Pattern A; High Pattern B. Reference Range: Pattern A.				
LDL PEAK SIZE		216.8	>222.9 Angstrom	Z4M
This test was developed and its analytical performance characteristics have been determined by Quest Diagnostics Cardiometabolic Center of Excellence at Cleveland HeartLab. It has not been cleared or approved by the U.S. Food and Drug Administration. This assay has been validated pursuant to the CLIA regulations and is used for clinical purposes. Relative Risk: Optimal >222.9; Moderate 222.9-217.4; High <217.4. Male and Female Reference Range: 216 to 234.3 Angstrom. Adult cardiovascular event risk category cut points (optimal, moderate, high) are based on an adult U.S. reference population plus two large cohort study populations. Association between lipoprotein subfractions and cardiovascular events is based on Musunuru et al. ATVB.2009;29:1975. For additional information, please refer to http://education.QuestDiagnostics.com/faq/FAQ134 (This link is being provided for informational/educational purposes only.)				
LDL SMALL		223	<142 nmol/L	Z4M
Relative Risk: Optimal <142; Moderate 142-219; High >219. Male Reference Range: 123 to 441 nmol/L; Female Reference Range: 115 to 386 nmol/L.				
LDL-CHOLESTEROL		103	<100 mg/dL (calc)	Z4M
Desirable range <100 mg/dL for primary prevention; <70 mg/dL for patients with CHD or diabetic patients with >= 2 CHD risk factors. LDL-C is now calculated using the Martin-Hopkins calculation, which is a validated novel method providing better accuracy than the Friedewald equation in the estimation of LDL-C. Martin SS et al. JAMA. 2013;310(19): 2061-2068 (http://education.QuestDiagnostics.com/faq/FAQ164)				
APOLIPOPROTEIN B	82		<90 mg/dL	Z4M
Reference Range <90 Risk Category: Optimal <90 Moderate 90-129 High > or = 130 A desirable treatment target may be <80 mg/dL or lower depending on the risk category of the patient including patients on lipid lowering therapies, patients with ASCVD, diabetes with >1 risk factors, Stage 3 or greater CKD with albuminuria, or heterozygous familial hypercholesterolemia. ApoB relative risk category cut points are based on AACE/ACE and ACC/AHA recommendations (Grundy SM, et al. 2019. doi:10.1016/j.jacc.2018.11.002; Handelsman Y, et al. 2020. doi:10.4158/CS-2020-0490).				



Patient Information	Specimen Information	Client Information
HANEY, MATTHEW DOB: 04/26/1993 AGE: 33 Gender: M Fasting: Y Patient ID: 5467CB51 Health ID: 8573004336378437	Specimen: ZD835935N Collected: 08/06/2026 / 10:41 PDT Received: 08/06/2026 / 19:36 PDT Reported: 08/22/2026 / 13:28 PDT	Client #: 73929412 DAMASCO, LEO

Reference Range/Comments

Analyte Name	In Range	Out Range	Reference Range	Lab
ARACHIDONIC ACID	12.3		8.6-15.6 % by wt	EZ
CHOL/HDL-C RATIO	3.5		<5.0 calc	Z4M
CHOLESTEROL, TOTAL	166		<200 mg/dL	Z4M
DHA	2.0		1.4-5.1 % by wt	EZ
DPA	0.8		0.8-1.8 % by wt	EZ
EPA	0.2		0.2-2.3 % by wt	EZ
HDL CHOLESTEROL	47		>39 mg/dL	Z4M
LINOLEIC ACID	19.6		18.6-29.5 % by wt	EZ
LIPOPROTEIN (a)	45		<75 nmol/L	Z4M
Risk: Optimal <75 nmol/L; Moderate 75-125 nmol/L; High >125 nmol/L. Cardiovascular event risk category cut points (optimal, moderate, high) are based on Tsimika S. JACC 2017;69:692-711.				
NON HDL CHOLESTEROL	119		<130 mg/dL (calc)	Z4M
For patients with diabetes plus 1 major ASCVD risk factor, treating to a non-HDL-C goal of <100 mg/dL (LDL-C of <70 mg/dL) is considered a therapeutic option.				
OMEGA-3 TOTAL	3.0		% by wt	EZ
OMEGA-6 TOTAL	35.3		% by wt	EZ
Quest Diagnostics measures a number of omega-6 fatty acids with AA and LA being the two most abundant forms reported.				
OMEGA-6/OMEGA-3 RATIO	11.7		3.7-14.4	EZ
TRIGLYCERIDES	71		<150 mg/dL	Z4M

PERFORMING SITE:

EN QUEST DIAGNOSTICS-WEST HILLS, 8401 FALLBROOK AVENUE, WEST HILLS, CA 91304-3226 Laboratory Director: THOMAS MCDONALD, MD, CLIA: 05D0642827
EZ QUEST DIAGNOSTICS/NICHOLS SJ, 33608 ORTEGA HWY, SAN JUAN CAPISTRANO, CA 92675-2042 Laboratory Director: IRINA MARAMICA, MD, PHD, MBA, CLIA: 05D0643352
Z4M CLEVELAND HEARTLAB INC, 6701 CARNEGIE AVENUE SUITE 500, CLEVELAND, OH 44103-4623 Laboratory Director: M. QASIM ANSARI, MD, CLIA: 36D1032987